Organizing the intersectoral response to complex and co-occurring social needs in primary care

Configuring networks adapted to the different profiles of patients in care



Eric Bernier, LL.M., M.Sc.
eric.bernier.1@umontreal.ca
PhD in public health · Health systems, organizations and policies · ESPUM, Université de Montréal

CO-SUPERVISED BY
Drs. Nadia Sourial and Géraldine Layani

01 Context

In primary care, social needs pile up

54 %
of patients have at least one unmet social need
Jih et al., 2023 · US data

351
different combinations of two or more needs
Kreuter et al., 2021 · US data

THREE DISTINCT NOTIONS

Co-occurrence is counted; complexity is described; the intersectoral response is organized within a territory.

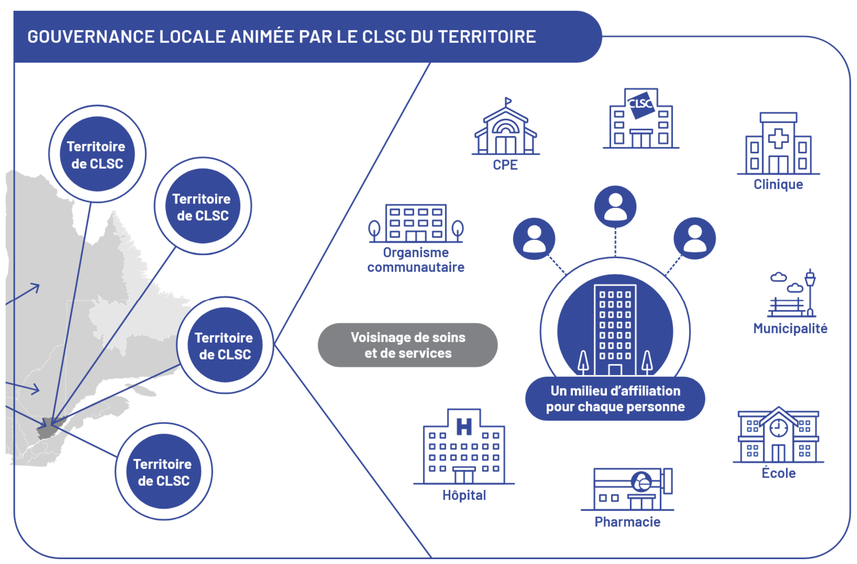
INCLUSION CRITERION
Co-occurrence
At least two social needs present at the same time.
Kreuter et al., 2021

DESCRIBED, NOT REQUIRED
Complexity
Accumulation of chronic diseases, disabilities, mental health problems and social vulnerability.
Kuluski et al., 2017

ORGANIZED WITHIN A TERRITORY
Intersectoral response
Actions by sectors other than health, carried out with it, on health or its determinants: CLSC, hospital, family medicine groups, pharmacies, community organizations, municipality, schools, childcare, private actors.
MSSS, 2017; Leppo et al., 2013

QUÉBEC PRIMARY CARE POLICY · MSSS, 2026

The CLSC territory becomes the reference unit



FOUR ROLES OF THE CLSC

Navigator · Coordinator · Intersectoral leader · Co-facilitator

"in a spirit of shared governance" · MSSS, 2026, p. 42 · figure: MSSS, 2026 (in French)

FICTIONAL CASE · COMMON THREAD

Ms. Protocol · 58
Who responds? With whom? Who coordinates?

Eviction notice → Food insecurity → Debts → Depression (clinical context)

THE GAP

The CLSC must coordinate. Yet for a given combination of needs, nothing says which partners to mobilize, how to link them, or who coordinates. **The planned programming is known, but not the networks actually formed around care pathways.**

RESEARCH QUESTION

How is the intersectoral response organized for the different profiles of patients with complex and co-occurring social needs cared for in primary care?

Aim: link each profile to...
A needs profile → a configuration of partners → and its coordinator

02 Methods

Setting: 3 contrasting CLSC territories in Lanaudière, a pioneering region in intersectoral action (intersectoral table) · multiphase design · O2 explanatory sequential mixed methods

Which profiles?
O1 · IDENTIFY
Scoping review
4–6 vignettes, validated by 6–8 stakeholders
Published studies · Recurring profiles · Vignettes
→ profiles turned into vignettes
MP *Her profile is found in the literature*

Whom to mobilize?
O2 · ANALYZE
CLSC ego networks
3 CLSCs · several respondents each
MP Whom to contact? Who coordinates?
→ one configuration per profile
MP *She becomes a vignette*

What really happens?
O2 · ANALYZE
Real pathways, interviews
2–3 real pathways per profile · 15–25 interviews
"Why are these partners so loosely connected?"
→ real network, gaps, explanations
MP *Her real pathway is compared with her map*

What to recommend?
O3 · RECOMMEND
Deliberative workshop
15–20 participants, incl. patient partners
Patient partners · Primary care · Community · Local governance · Municipal, private
→ recommendations per profile
MP *Her case is deliberated in the workshop*

Integrate
32 months · 2026 → 2029
Joint display by profile

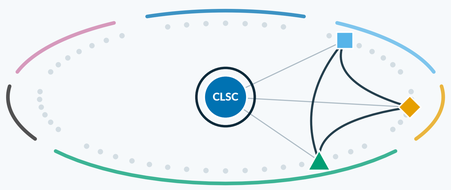
Profile	Network	Why	Recs
A			
B			
C			

MP *Guidance for her profile*

03 Expected results

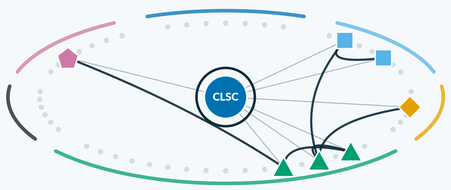
Fictional examples · **in each CLSC, the ego network changes with the profile**

A Housing-dominant
eviction notice



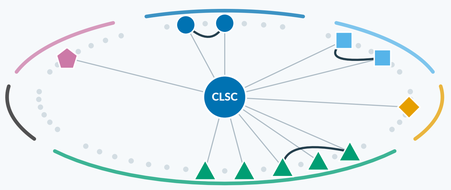
3 partners, all linked · the CLSC coordinates

B Ms. Protocol
housing · food · debts



7 partners, still linked · the CLSC coordinates

C Accumulation + depression
same needs, plus depression



11 partners, loosely linked · no coordinator

CLSC (ego) · designated coordinator · tie between partners · organization not mobilized
Health and social services · Primary care, pharmacies · Community · Municipal · Education, early childhood · Employment, justice, private

H1 Sectors change with the profile → *sector composition*
H2 A common core, a specific periphery → *shared partners*
H3 More partners, but less connected → *size and density*
H4 Does the CLSC navigate, coordinate? → *designated coordinator*
H5 Does gender change the response? → *female, male versions*
≠ Real or potential: what gap? → *real pathways, vignettes*

04 Impacts and outcomes

Not a universal "ideal network": **guidance for each profile.**

For people
A response tailored to their combination of needs

For the CLSC and its partners
Guidance: whom to mobilize, how to connect, who coordinates

For primary care policy
A baseline; recommendations aligned with the CLSC's four roles

For research
A profile–network hypothesis; the gap between potential and real networks

Dissemination: feedback to the 3 territories · protocol on OSF · articles

French, colour-blind and audio versions
Français · daltonisme · audio

Comment, suggest, get in touch
Readings · contacts · methodological ideas

References: Creswell & Plano Clark, 2017 · Gale et al., 2013 · Jih et al., 2023, BMC Primary Care · Kreuter et al., 2021, Population Health Management · Kuluski et al., 2017, Int J Integr Care · Leppo et al., 2013 · Leutz, 1999 · MSSS, 2017; 2026, Québec primary care policy · Peters et al., 2020 · Thorne, 2016 · Colour-blind version: Okabe-Ito colours and one shape per sector